Nontherapeutic Circumcision, Ethics and Law: Venditor caveat

In “Trends in Circumcision Among Newborn Males in the US”(1) Yang and co-authors reference the 2012 American Academy of Pediatrics (AAP) circumcision statement that expired in 2017(2). Since the AAP intentionally did not reaffirm, renew or replace that policy, we question the intellectual honesty of the authors.

We further question their disingenuous claim that the AAP “recommended” newborn circumcision. False! The AAP stated “Although health benefits are not great enough to recommend routine circumcision for all male newborns, the benefits of circumcision are sufficient to justify access to this procedure.” This merits closer scrutiny.

The AAP, like most pediatric associations worldwide, did not “recommend” circumcision on health grounds, but cleverly referred to other vague “benefits,” presumably nonmedical (social) ones. Circumcision Task Force and Bioethics Committee member Andrew Freedman subsequently said as much this(3), raising the question of why physicians should even be acting as agents of a social custom. Incidentally, the 1995 Bioethics Committee determined that ‘proxy consent’ poses serious problems for pediatric healthcare providers, stating such providers have legal and ethical duties to their child patients to render competent medical care based on what the patient needs not what someone else expresses.

Many physicians intentionally abuse the term “phimosis” (the natural, nonpathological condition in infancy) as a “diagnosis” to justify circumcision and insurance reimbursement, thereby disregarding their legal and ethical duties to render care based on what their child patient needs, deferring instead to what someone else expresses (parents).

This violate numerous tenets of the AMA Code of Medical Ethics, as well as the Hippocratic Oath to “do no harm.” They rely upon specious and spurious “medical” rationalizations (proffered by Yang and co-authors) to justify a nontherapeutic, nonconsensual violation of children’s bodily integrity and genital autonomy. They persist in passing the buck to parents (often uninformed, misinformed or emotionally influenced) who then decide to impose this permanent, irreversible and harmful sexual surgery(4) on their child before he’s able to decide for himself how much of his genitals he gets to keep.

Physicians must uphold their legal and ethical duties to child patients by abstaining from needless childhood genital cutting, regardless of sex.(5) If they cannot find the ethical integrity to resist pressure from parents, or their medical peers, to defend an open future for the child, then the courts must rein in physician negligence by ensuring that all children be protected from these harmful surgical interventions.

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